

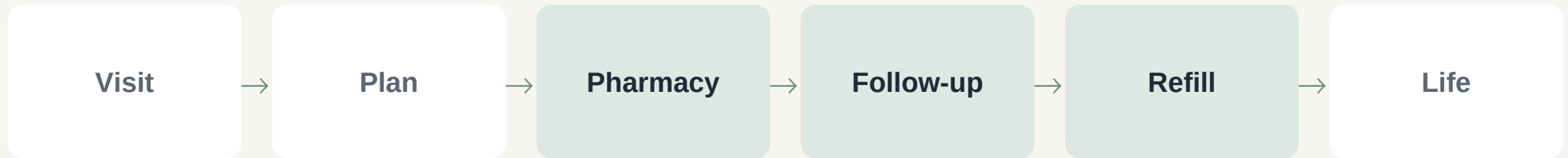
Siya Health®

**We started with patients.
Not software.**

Physician-led ADHD + primary care telehealth · four states (CA · TX · PA · FL)

The problem

**Patients weren't failing to find care.
Care was failing to follow them.**



Everyone optimizes the first visit.

Care actually breaks down at Pharmacy · Follow-up · Refill.

The solution

So we built the missing pieces ourselves.

Physicians → Operators → Systems builders

Multi-state licensure

CA · TX · PA · FL — providers credentialed and compliant in every state we serve.

Offshore clinical operations

Hired, trained, and QA'd from scratch — scheduling, pharmacy coordination, patient follow-up.

Standardized provider workflows

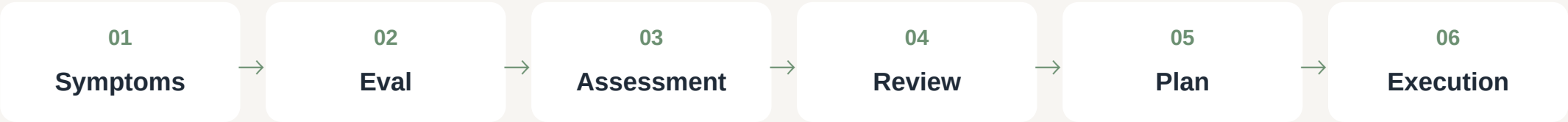
Protocols and SOPs shared across all providers and states — care doesn't depend on memory

Product

The care journey we run today — end to end.

What we deliver today

End-to-end ADHD care, held together by our protocols and clinical judgment — not by any single vendor tool.



Escalation loop

When care stalls — pharmacy hold, side effects, missed follow-up, refill risk — we escalate, re-enter Review → Plan → Execution, and close the loop. Continuity is the product of the process.

■ Escalation feeds back into Review — care does not end at the first plan.

What we’re building

Concept — in design, informed by workflows already validated in our practice.

Care Timeline → Provider Workspace → Refill / Follow-up queue — software that encodes this journey.

Traction

Revenue nearly 3× since February.
Cash-flow positive.

2,000+

Patients treated

1,000+

ADHD evaluations

150+

Recurring members

600+

Reviews · 4.8★

4 states

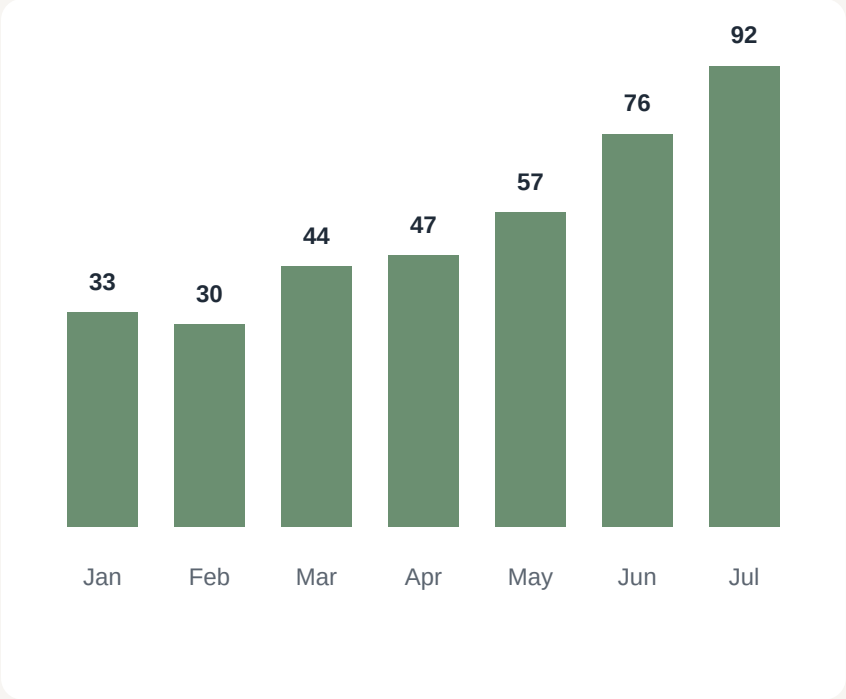
CA · TX · PA · FL

Cash-flow+

Operations

\$378k+ gross 2026 YTD (Jan–Jul) · founder-confirmed books.

Monthly gross revenue — 2026



\$ thousands / month · gross

Where we are, honestly

Our growth engine today is founder-led — and it works.

Proof

Founder-led relationships and networking — on top of a full clinical load — fill all providers' panels, not just the founder's own.

Risk

That is a key-person dependency.
We say so plainly.

Fix

This round funds clinical capacity and redundancy, so proven acquisition runs without routing every relationship through one person.

Proof + risk + fix — not a disclosure buried in an appendix.

Cash-pay, recurring, no insurance risk.

\$149

Initial evaluation

One-time, self-pay

\$79 / mo

Ongoing care — non-controlled

Recurring membership

\$149 / mo

Ongoing care — controlled

Recurring membership

Subscription-style continuity: 150+ recurring members and growing.

Roughly \$600 per patient-year in professional fees (bottom-up, competitor-benchmarked).

Cash-pay means no payer contracts, no reimbursement risk, and revenue from day one.

15.5M diagnosed adults. \$2.9B serviceable today.

Layer	Definition	Size
TAM	U.S. adult ADHD clinical services (professional fees)	~\$10B
SAM	Adult ADHD virtual care (telehealth-willing, cash-pay comps)	~\$2.9B
SOM	Year-3, capacity-derived (~40 clinician FTEs)	~\$16M

Built bottom-up: 15.5M diagnosed U.S. adults (CDC) · ~half already use telehealth for ADHD · ~\$600/patient-year professional fees.

No inflated blended TAM. Full sizing workbook available in diligence.

Competition

Different job. Not a feature checklist.

Episodic telehealth	Optimizes first-visit volume, not continuity
Generic EHR tools	Records history, doesn't guide next action
Point cognitive tools	One-time score, not ongoing care
Traditional psychiatry	Access-limited, waitlist-bound
Siya	Continuity, evaluation through refill — 2,000+ patients

No feature matrix. No “everyone top-right” quadrant. The difference is the job we do.

A process — not a channel wishlist.

01 · Today

Primary engine

Founder-led relationships and networking fill all providers' panels — not just the founder's.

02 · Live test

Texas Search

Paid Search test live in Texas — [confirm conversion count from Google Ads before upload]

03 · Transition

This round's job

Add clinical capacity, then move from founder-led fill to systematic acquisition — reproduce Search TX → CA.

04 · Measurement

No guessing

Full CAC and payback period confirmed by August 30, 2026.

Why now

The playbook is proven. The environment rewards how we practice.

Texas

Search conversions proven — a repeatable acquisition playbook exists

California

Revenue growing — next market for the same playbook

Regulation

Environment rewards protocolized, auditable care over low-touch mills

Discipline

SOPs, QA, and multi-state compliance already in place

We're raising \$500k to scale a working practice.

01 Clinical capacity & redundancy

More providers and coverage — directly reduces founder key-person risk.

02 Scale proven acquisition

Texas Search converts today. Reproduce the same playbook in California.

03 Working capital buffer

Operate from strength, not desperation.

Objective: 18–24 months of runway to prove the playbook in a second state and remove key-person dependency.

Founders

Physicians who built the practice before asking for capital.



Dr. Sneh Pandey

Founder · CEO

- Board-certified physician
- Tens of thousands of patient encounters — primary care, obesity, hospital medicine, ICU
- Built multi-state practice to 2,000+ patients, cash-flow positive, bootstrapped



Dr. Swati Pandey

Co-founder

- Board-certified physician
- Co-built clinical protocols and provider workflows across four states
- Leads clinical quality and patient-experience standards

**Years spent learning how to deliver better care.
This round scales what already works.**

Seed round

\$500,000

Clinical capacity & redundancy · Proven acquisition, next state · Working capital

Siya Health®

Let's talk.

Dr. Sneh Pandey · Founder & CEO

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